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सत्यमेव जयते

Department of Health Research
Ministry of Health and Family Welfare,
Government of India

Standard Treatment Workflow (STW)

KANGAROO MOTHER CARE

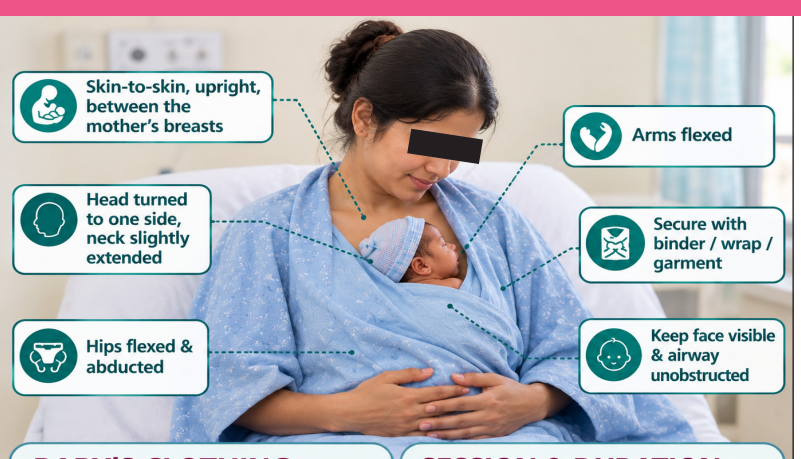
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Kangaroo Mother Care (KMC) is early, continuous and prolonged skin-to-skin contact between a preterm or low birth weight newborn and the mother (or an alternate caregiver), combined with exclusive breastmilk feeding, early discharge when appropriate, and structured follow-up.

Start KMC for every preterm(<37 weeks) or low birth weight baby (<2500g) soon after birth

Except if the baby is unable to breathe spontaneously after resuscitation, is in shock or needs mechanical ventilation

KANGAROO POSITION



BABY'S CLOTHING
- Wear cap to prevent heat loss
- Socks to keep feet warm
- Diaper or nappy

SESSION & DURATION
- Each session at least **1 hour**
- Aim for **24 hours** while providing **AT LEAST 8 hours**
- Record cumulative duration per 24 h

KMC PROVIDER CAN BE

1. Mother (Preferred) - the primary KMC provider.
2. Father "(Alternate) - preferred alternate"; identify & prepare before birth
3. Another trained adult family member (Backup) when the mother is temporarily unavailable
4. Return to maternal KMC as soon as the mother is able

KANGAROO NUTRITION

- Initiate breastfeeding as early as possible
- Support exclusive breastfeeding; encourage direct feeds
- Expressed breast milk if direct feeding not possible
- Express and feed in KMC position
- Use Cup/spoon/paladai until effective feeding established
- Continue OG/NG tube feeds if indicated
- Provide lactation support

KANGAROO DISCHARGE

- Discharge when the neonate meets discharge criteria as per unit policy*
- Counsel for continuation of KMC while going home
- Continue KMC at home
- Refer to discharge and follow-up STW module

ENSURE BEFORE STARTING KMC

- Appropriate KMC bed/wrap available
- Clean, comfortable, private area secured
- Documentation & monitoring chart available
- Trained healthcare provider available
- Mother/caregiver counselled
- Baby is upright or at least at 30 degrees

MONITORING

AT INITIATION

Monitor temperature after starting KMC
- Assess breathing & colour
- Assess positioning & airway
- Assess feeding

DURING CONTINUATION
- Monitor temperature regularly
- Assess feeding & weight
- Track & document KMC duration

KMC IN SICK BABIES
- Monitor heart rate, SpO_2 and respiratory rate/breathing and colour

KMC DURING TRANSPORT

- Continue KMC at discharge to Home
- Continue during intra- & inter-facility transport where feasible
- Confirm clinical stability before transporting in KMC position
- Secure safely with binder/wrap/KMC bag
- Accompany with trained provider when indicated
- Minimize interruption before, during & after transport

CONTINUE KMC WHILE ON

- Oxygen therapy
- CPAP
- IV fluids or medications
- OG/NG tube feeding
- Phototherapy (interrupt only during treatment, resume immediately)
- Twins: individually or together, depending on stability and caregiver availability

HOME KMC COUNSELLING

- Continue KMC as long as possible
- Continue exclusive breastfeeding
- Encourage family participation
- Attend scheduled follow-up
- Seek care if "danger signs" develop
- Go to Neonatal Triage STW Module

FOLLOW-UP VISITS

- Assess weight gain, length and head circumference
- Assess breastfeeding & feeding adequacy
- Assess KMC practice
- Screen for danger signs
- ROP screening, hearing screening, neurodevelopmental assessment as required
- Reinforce KMC & feeding counselling each visit

DO's

- Start KMC early in every eligible preterm or low-birth-weight baby; continue as long as possible
- Ensure correct positioning, a clear airway, and secure support; monitor temperature, breathing, and feeding
- Support exclusive breast milk feeding and counsel families to continue KMC at home with scheduled follow-up

DON'Ts

- Do not initiate KMC in babies unable to breathe spontaneously after resuscitation, in shock, or requiring mechanical ventilation
- Do not routinely stop KMC for oxygen, CPAP, IV therapy, or tube feeding
- Do not delay KMC because the mother is unavailable; involve a trained alternate caregiver

KEY PERFORMANCE INDICATORS (KPIs)

$$\text{KMC coverage among eligible neonates} = \frac{\text{No. of neonates initiated on KMC}}{\text{Total eligible neonates during that period}} \times 100 \quad \text{(Target: ≥90\%)}$$

$$\text{Percentage of eligible neonates getting > 8 hours of KMC} = \frac{\text{Neonates receiving KMC for ≥8 h per 24h}}{\text{All neonates receiving KMC}} \times 100 \quad \text{(Target: ≥80\%)}$$

MOTHER CAN SLEEP, STAND AND WALK IN KMC POSITION



EARLY, CONTINUOUS SKIN-TO-SKIN KMC SAVES NEWBORN LIVES

RELATED INFORMATION

Breastfeeding in KMC Position

Expressing Milk in KMC Position

Initiating KMC after Birth

Placing in KMC Position

Postnatal Counselling for KMC

ABBREVIATIONS

CPAP - Continuous Positive Airway Pressure
EBM: Expressed Breastmilk
KMC: Kangaroo Mother Care
LBW: Low Birth Weight
OG/NG - Orogastric/Nasogastric
ROP- Retinopathy of Prematurity
SpO_2- Peripheral Oxygen Saturation

REFERENCES

1. World Health Organization. Kangaroo mother care: a clinical practice guide. Geneva: WHO; 2025.
2. Government of India. Operational guidelines for Kangaroo Mother Care (KMC) and optimal feeding of low birth weight infants. New Delhi: Ministry of Health and Family Welfare; 2014.
3. Ministry of Health and Family Welfare, Government of India. Facility Based Newborn Care (FBNC): Participant Manual. New Delhi: MoHFW; 2023.

This STW has been prepared by national experts of India with feasibility considerations for various levels of healthcare system in the country. These broad guidelines are advisory, and are based on expert opinions and available scientific evidence. There may be variations in the management of an individual patient based on his/her specific condition, as decided by the treating physician. There will be no indemnity for direct or indirect consequences. Kindly visit the DHR portal (**stw.icmr.org.in**) for more information.©Department of Health Research, Ministry of Health & Family Welfare, Government of India.